

v9 Data-Quality Review

Carrier rate-data comparison — v9 (pre-production) vs. v8_2

Internal results summary · June 2026

Bottom line. v9 increases the share of spend backed by real rate data for three of the four carriers, and the plans remain priced at near-parity. The one area that moved backward — and the focus for follow-up — is outpatient-facility coverage for Cigna and UHC.

Data quality improved from v8_2 to v9

The Green/Yellow (MRF-backed) share of spend — the portion of rates grounded in real machine-readable-file (MRF) data rather than imputed — rose for three of the four carriers nationally:

Carrier	G/Y backed, v8_2 → v9
Aetna	73.4% → 77.1% (+3.7)
BCBS	59.9% → 64.2% (+4.3)
UHC	59.3% → 63.2% (+3.9)
Cigna	60.7% → 61.0% (+0.3)

Aetna, BCBS, and UHC each gained roughly 4 points of MRF-backed coverage. Cigna effectively held flat — and the reason it stood still while the others improved is specific to one service category, covered below.

Carriers price at near-parity

Nationally the four anchor carriers land within about 1.8% of each other on blended spend: Aetna marginally lowest (−0.4% vs. BCBS), then BCBS, Cigna, and UHC highest (+1.4% vs. BCBS). The pattern holds across settings — indexed to BCBS = 100, every carrier sits between 98 and 103 in inpatient, outpatient, and professional, with UHC the only consistent (slight) high mark, most visible in professional (103). These plans are priced almost identically; there is no material cost outlier among them.

Quality varies more by carrier than price does

Where the carriers differ is in how well their rates are supported. Spend breaks into four sources — carrier MRF and hospital MRF (real rate data) versus red and missing (imputed) — summing to 100.0%:

Carrier	carrier-MRF	hospital-MRF	MRF-backed	red	missing	imputed
Aetna	66.4%	10.6%	77.1%	10.0%	12.9%	22.9%
BCBS	49.8%	14.4%	64.2%	14.2%	21.6%	35.8%
UHC	48.3%	14.9%	63.2%	12.6%	24.2%	36.8%
Cigna	46.1%	14.8%	61.0%	15.4%	23.7%	39.0%

Aetna's rates are far better grounded — 77.1% MRF-backed versus 61.0–64.2% for the others, which lean roughly 1.6x harder on imputation. So while the carriers price the same, that equivalence rests on much firmer evidence for Aetna than for Cigna or UHC, and any head-to-head comparison should account for that asymmetry.

Where the v9 gains and gaps are, by service category

The national improvement is not uniform across service categories — and the category view explains the carrier-level differences above. The table shows G/Y-backed share, v8_2 → v9:

Service category	Aetna	BCBS	Cigna	UHC
Inpatient	71.8% → 71.6% (-0.2)	74.1% → 79.7% (+5.6)	73.0% → 76.0% (+3.0)	79.6% → 81.2% (+1.7)
Outpatient facility	67.9% → 73.7% (+5.8)	44.7% → 49.7% (+5.0)	43.1% → 40.4% (-2.7)	43.7% → 39.4% (-4.3)
Professional	79.5% → 82.9% (+3.4)	66.1% → 68.6% (+2.5)	70.0% → 70.8% (+0.8)	62.6% → 74.4% (+11.8)
All	73.4% → 77.1% (+3.7)	59.9% → 64.2% (+4.3)	60.7% → 61.0% (+0.3)	59.3% → 63.2% (+3.9)

Inpatient and professional improved broadly — nearly every carrier gained in both, and no carrier lost ground in either. UHC's professional gain (+11.8) is the largest single move in the matrix.

Outpatient facility is the divergence point. It improved solidly for Aetna (+5.8) and BCBS (+5.0) but *declined* for Cigna (-2.7) and UHC (-4.3) — the only two negative cells in the table. That single category is why Cigna nets flat overall:

its inpatient and professional gains are cancelled by its outpatient-facility loss. UHC shows the same outpatient decline but still nets positive because its professional gain is so large.

Review flags and remaining churn

The needs-review flag fires most in outpatient (92.1%) and inpatient (85.7%), less in the combined All bucket (61.4%), and least in professional (17.6%). The clearest data-grounding concern is outpatient facility, which carries the lowest G/Y in the matrix; inpatient, despite its high flag rate, stays well-grounded overall because hospital MRF supplies usable rates where carrier coverage is thin. At the state-by-carrier matrix level, flags reflect data-source conditions rather than competitive positioning, so the matrix reads as a map of data confidence, not of price.

The remaining version-over-version change is moderate: 73.6% of cells are stable or clean, and among the rest more improved (2,665) than regressed (1,748). Large spend swings (over 10.0%) affect 7.8% of cells, concentrated in inpatient and professional, where fewer underlying rates make the blended figure more sensitive to small changes. Regressions concentrate in outpatient and spread evenly across all four carriers, so no single carrier drives them.

Spots to review

- **Cigna and UHC outpatient facility** — the priority. Both declined version-over-version *and* now sit at the lowest G/Y in the matrix (40.4% and 39.4%, roughly 60% imputed). The mechanism differs: UHC's slip is rising *missing* spend (rates with no MRF at all), while Cigna's is rising *red* (low-confidence imputation). The question to answer is why outpatient facility lost ground for these two when it improved for Aetna and BCBS.
- **UHC professional (+11.8)** — worth a check in the other direction. A single-category jump that large is worth confirming as a real data gain before it is quoted as a win.

A practical validation starting point: sample the low-G/Y outpatient-facility cells for Cigna and UHC, plus the UHC professional gain, in the higher-population metros.

Source: v9_DEV4 version-comparison extract — v9 (preprod_v9_noper) vs. v8_2, all-setting (TOTAL) bucket at national, state, and MSA grain. Figures are percent of spend; G/Y = Green/Yellow = MRF-backed share.